4.4 Myocardial Infarction*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The chest pain is crushing and goes to my left arm." • Rates pain 9/10 **Objective** • Sweating and pale • ST elevation on ECG • Raised troponin	**ACTUAL** Acute pain related to myocardial ischemia and necrosis as evidenced by crushing chest pain, diaphoresis and ECG changes.	• Pain relieved within 30 minutes of treatment.	• Assess pain and record an ECG straight away. • Give aspirin, nitroglycerin and IV morphine as prescribed. • Give oxygen if SpO_2 is below target. • Prepare for reperfusion by PCI or thrombolysis, as ordered. • Keep him on bed rest in a calm setting.	• Met once pain is relieved.
Subjective • "I feel weak and dizzy." **Objective** • BP 94/60 mmHg, HR 112/min • Cool skin • Urine output falling	**ACTUAL** Ineffective peripheral tissue perfusion related to reduced myocardial contractility as evidenced by hypotension, cool skin and reduced urine output.	• Systolic BP above 90 mmHg and urine at least 0.5 mL/kg/hour.	• Keep him on continuous ECG; record vital signs every 15 minutes at first. • Measure urine output hourly. • Listen to heart and lungs for new murmurs or crackles. • Give medicines as prescribed.	• Achieved when hemodynamics are stable.
Subjective *No specific complaint* **Objective** • Acute myocardial injury	**RISK** Risk for decreased cardiac output as evidenced by risk factors: post-infarction dysrhythmias.	• Any dysrhythmia is detected and treated quickly.	• Keep a defibrillator and emergency drugs close by. • Follow electrolytes, especially potassium and magnesium. • Report new dysrhythmias at once.	• Outcome reached if no dysrhythmia goes untreated.

*References for this care plan: [11], [17], [32], [35], [128]. Full entries are listed in the References section of the CPMN book.